

PALM MUTUAL OF FLORIDA
MEDICAL COVERAGE GUIDELINE
Policy PM-02-28000-09
Extracranial Carotid Revascularization

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Original effective date	06/01/2022
Reviewed	08/01/2025
Revised	08/20/2025
Lines of business	Commercial and Medicare Advantage, as a synthetic test policy only

Description

Stroke referable to the extracranial internal carotid artery is treated by removing or stenting the plaque when the stenosis is severe and the patient has had a recent ipsilateral ischemic symptom. This policy covers carotid endarterectomy and cervical carotid stent placement with distal embolic protection. It does not cover intracranial stenting, stent placement without embolic protection, or revascularization of a patient who has not had ipsilateral symptoms.

Position statement

Carotid stent with embolic protection, CPT 37215

Transcatheter stent placement in the cervical carotid artery with distal embolic protection (CPT 37215) meets medical necessity when **all** of the following are documented:

CTA, contrast MRA, duplex using NASCET-style measurement, or catheter angiography within 6 months shows ipsilateral extracranial internal carotid stenosis of at least 70 percent by NASCET criteria, and the artery is not occluded. The report must state laterality and that the lesion is cervical, not intracranial; **AND**

An ipsilateral hemispheric TIA, amaurosis fugax, or nondisabling ischemic stroke in that carotid territory occurred within 6 months of the request. A carotid bruit, dizziness, or a scan done for screening does not satisfy this criterion; **AND**

The operative plan states that a distal embolic protection device will be used.

Carotid endarterectomy, CPT 35301

Carotid endarterectomy by neck incision, including patch angioplasty when performed (CPT 35301), meets medical necessity when **all** of the following are documented:

Imaging within 6 months, as defined in the stent pathway, shows ipsilateral extracranial internal carotid stenosis of at least 70 percent by NASCET criteria and the artery is not occluded; **AND**

An ipsilateral hemispheric TIA, amaurosis fugax, or nondisabling ischemic stroke in that territory occurred within 6 months of the request.

Embolic protection is not required for endarterectomy. Choosing endarterectomy rather than a stent does not remove the requirement for a recent ipsilateral ischemic symptom.

Exclusions and non-covered services

Exclusion. Asymptomatic carotid stenosis does not meet medical necessity under either pathway, including stenosis of 80 percent or more found on screening. Absence of symptoms is a failed criterion, not a missing document.

Exclusion. A complete occlusion of the internal carotid artery is not a stent or endarterectomy target under this policy.

Investigational. Cervical carotid stent placement without distal embolic protection (CPT 37216) does not meet medical necessity. Intracranial angioplasty or stenting, vertebral artery stenting, and acute stroke thrombectomy are outside this policy. A code that is not in the coding table is not covered here.

Billing and coding

Code	Description	Coverage
37215	Transcatheter cervical carotid stent, with angioplasty when performed, with distal embolic protection	Covered if criteria met
37216	Transcatheter cervical carotid stent without distal embolic protection	Investigational
35301	Thromboendarterectomy, including patch graft if performed; carotid, by neck incision	Covered if criteria met

Diagnosis codes that support medical necessity when the criteria above are also met include I65.21 and I65.22 (occlusion and stenosis of the carotid artery), G45.1 (carotid artery syndrome), G45.3 (amaurosis fugax), and the corresponding cerebral infarction codes when the stroke is nondisabling and within 6 months.

Documentation to submit

Neurology or vascular note describing the symptom, the date it occurred, the side of the body or the eye, and whether a deficit remains.

Imaging report dated within 6 months, with percent stenosis by NASCET, laterality, and a statement that the artery is patent rather than occluded.

For CPT 37215, a sentence in the operative plan that distal embolic protection will be used. A plan that omits protection is a 37216 request and is investigational.

The order with one CPT code. Do not infer a stent from the word intervention.

Definitions

Ipsilateral means the symptom is in the territory of the artery to be treated: right carotid stenosis with left-body hemispheric symptoms or right-eye amaurosis, and the reverse for the left carotid.

NASCET compares the narrowest residual lumen with the normal internal carotid artery distal to the bulb. A visual estimate that does not state NASCET criteria does not establish the percent threshold.

Nondisabling means the patient is a candidate for elective revascularization. This policy is not an acute-thrombectomy policy.

References

Published papers are named so this guideline has a bibliography. This policy is not those documents.

Brott TG, et al. 2011 ASA/ACCF/AHA/AANN/AANS/ACR/ASNR/CNS/SAIP/SCAI/SIR/SNIS/SVM/SVS guideline on the management of patients with extracranial carotid and vertebral artery disease. *Circulation*. 2011;124:e54-e130.

AbuRahma AF, et al. Society for Vascular Surgery clinical practice guidelines for management of extracranial cerebrovascular disease. *J Vasc Surg*. 2022;75:4S-22S.

North American Symptomatic Carotid Endarterectomy Trial Collaborators. Beneficial effect of carotid endarterectomy in symptomatic patients with high-grade carotid stenosis. *N Engl J Med*. 1991;325:445-453.